

Metropolitan City Hospital - Inpatient Service
DEATH SUMMARY

ATTENDING PHYSICIAN: Dr. Benjamin Carter, MD

HISTORY OF PRESENT ILLNESS:

Mr. Green was a 78-year-old gentleman with a past medical history significant for severe COPD (on home oxygen), coronary artery disease, and hypertension. He was diagnosed with de novo Acute Myeloid Leukemia on June 12, 2023. Bone marrow biopsy at that time revealed 85% myeloblasts and molecular testing was positive for a **TP53 mutation**, conferring an adverse-risk prognosis per ELN 2022 criteria.

Given his age and significant comorbidities, he was deemed not to be a candidate for intensive induction chemotherapy. He was started on a less intensive regimen of **Venetoclax plus Azacitidine** on June 20, 2023. He initially tolerated this therapy well and achieved a morphologic leukemia-free state after two cycles.

However, in mid October 2023, his routine bloodwork showed rapidly declining cell counts. He was admitted to the hospital on October 18, 2023, with severe fatigue, dyspnea, and pancytopenia.

HOSPITAL COURSE:

Upon admission, his CBC showed a WBC of 1.8 K/uL with 75% circulating blasts, Hemoglobin of 6.1 g/dL, and Platelets of 9 K/uL. A repeat bone marrow biopsy confirmed aggressive relapse of his disease.

The patient's case was discussed at length with him and his family. Given the highly aggressive nature of his TP53-mutated leukemia, his rapid relapse on first-line therapy, and his poor performance status (ECOG 4), it was determined that further anti-leukemic therapy would be futile and likely cause more harm than benefit.

The decision was made, in accordance with the patient's expressed wishes, to transition to comfort-focused care. The Palliative Care team was consulted and assumed primary management. He was kept comfortable with parenteral opioids for bone pain and received platelet transfusions to prevent bleeding. Over the next five days, his condition declined. He became progressively somnolent and passed away peacefully on the afternoon of October 23, 2023, with his family at his bedside.

CAUSE OF DEATH:

- **Primary:** Complications of Refractory Acute Myeloid Leukemia
- **Contributing:** Respiratory Failure

No autopsy was requested. The coroner was notified.

PATIENT: GREEN, George

MRN: AML031

DOB: 06-Jun-1945

ADMISSION DATE: 18-Oct-2023

DATE OF DEATH: 23-Oct-2023 at 14:50

PRIMARY DIAGNOSIS: Acute Myeloid Leukemia, Relapsed/Refractory

Dr. Benjamin Carter, MD

Hematology-Oncology

Dictated: 23-Oct-2023

Transcribed: 24-Oct-2023